NCCP 00432 · clinician draft

AC → weekly paclitaxel + weekly trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

HER2-only continuation

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Routine prophylactic antiemetics and paclitaxel premedicines are not carried into this phase. Treat prior nausea / infusion reactions under an individual plan.
- Continue HER2 cardiac monitoring; ending paclitaxel does not end echo surveillance.
- NCCP 00432 transitions after 12 weekly combination doses to trastuzumab 6 mg/kg every 21 days under regimen 00200, to complete one total year. Confirm the prescribed schedule and missed-dose rules; do not reuse weekly loading logic for 21-day continuation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00432 v5 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-23 · SACTCheck v0.84.17 · Regimen-level drafts; prescriber and oncology-pharmacy verification required